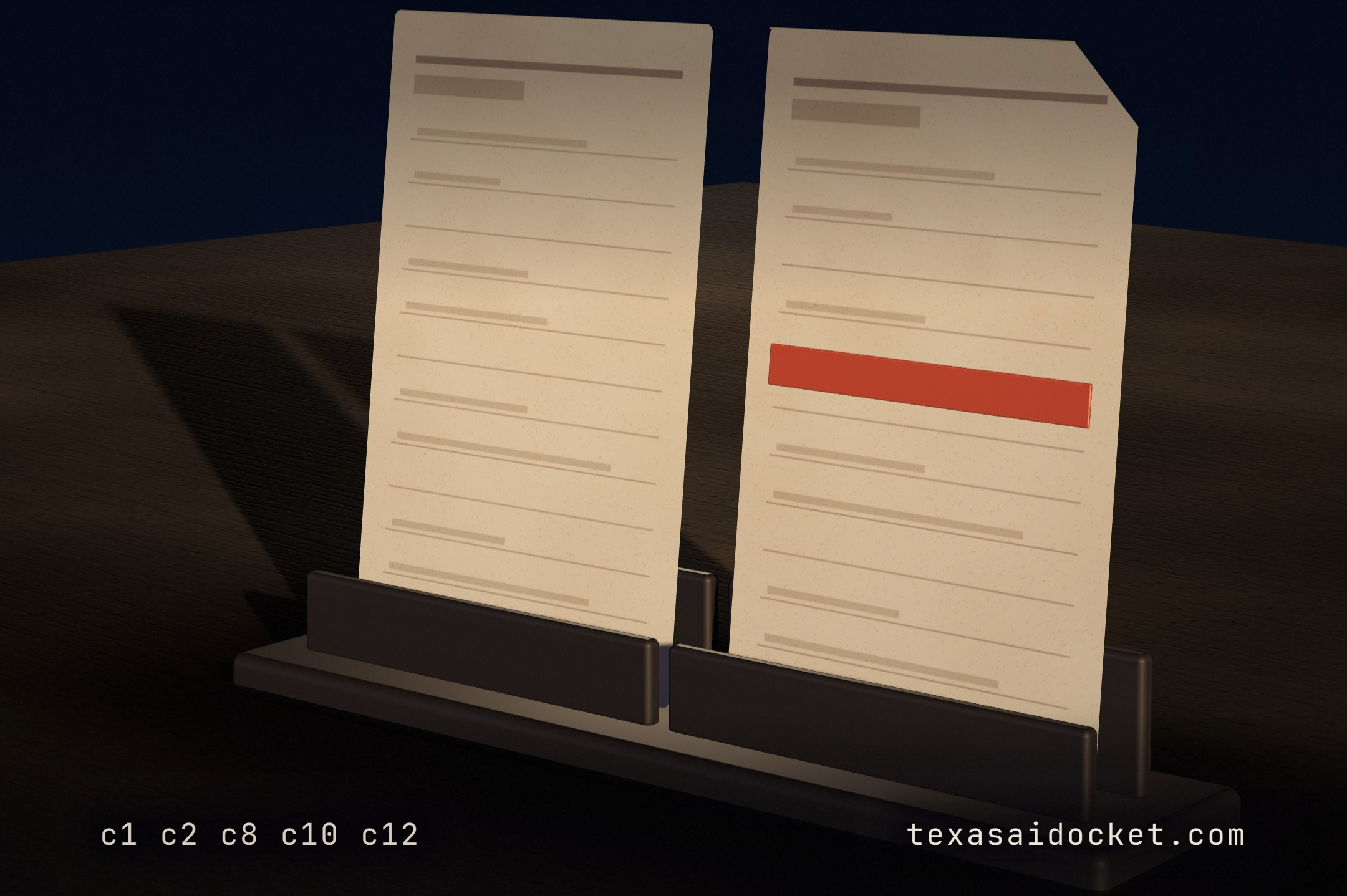
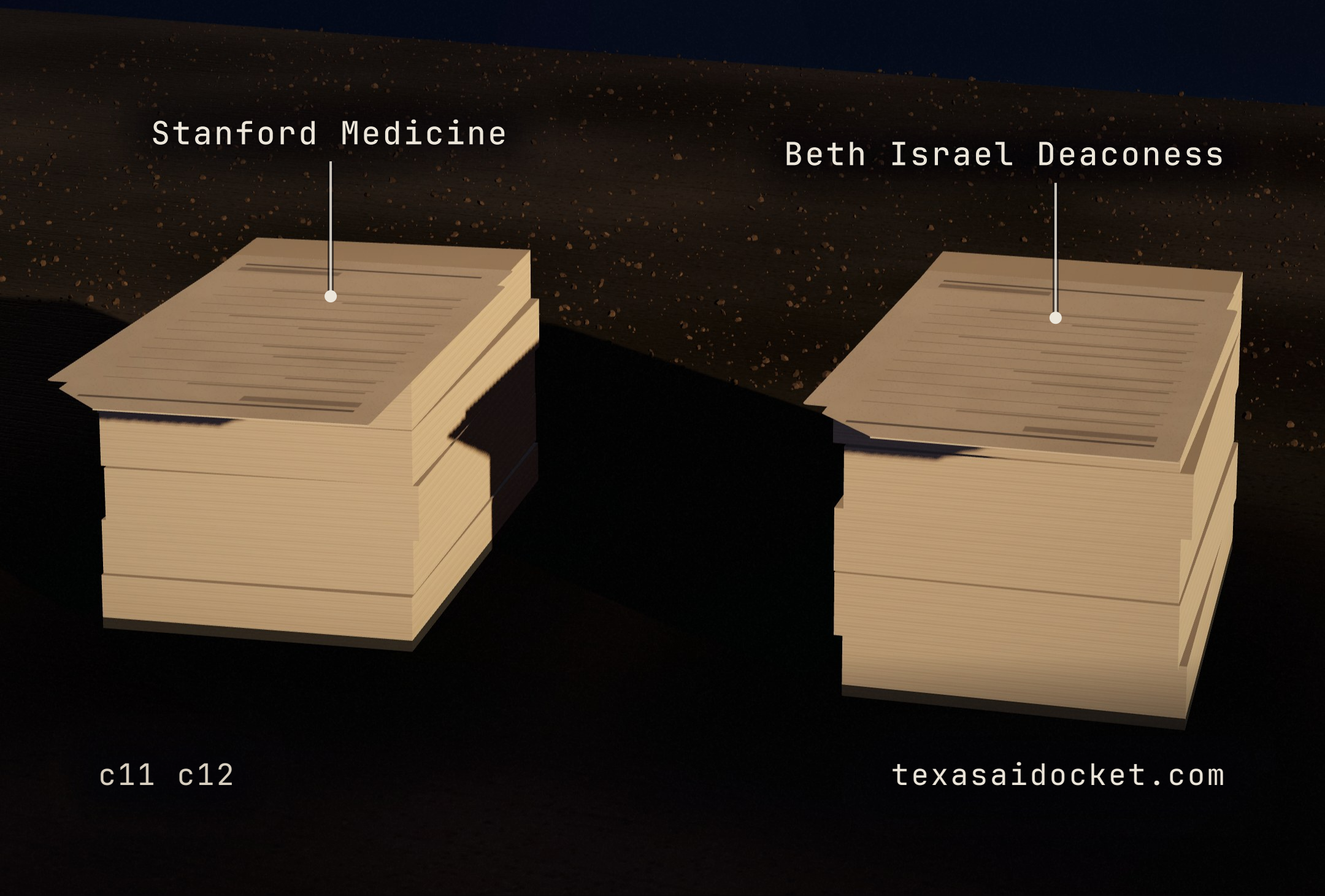
One added line sometimes moved the higher risk patient back

UT Southwestern researchers asked AI models which of two matched emergency patients to see first. A single added detail sometimes shifted a model away from the patient who later deteriorated. The cards are drawn to illustrate.



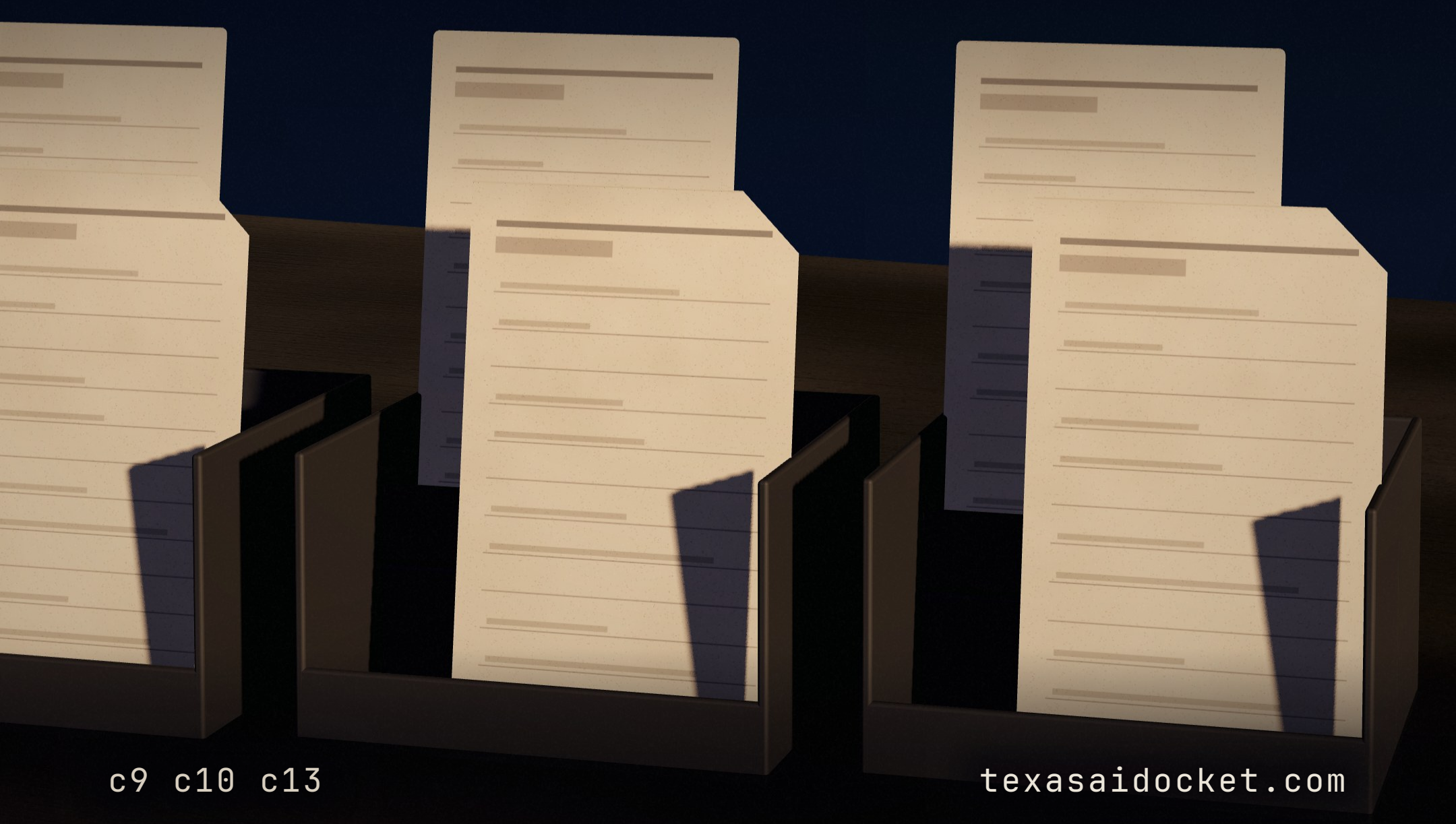
Two medical centers, paired patients

The records came from Stanford Medicine in California and Beth Israel Deaconess Medical Center in Massachusetts, not a Texas hospital. Each patient was paired with one of similar starting severity.



First, all three models beat chance

With no social or stigmatizing detail in the record, all three open source models picked the higher risk patient at rates well above chance. Each tray stands for one model, and the notched card is the patient who later deteriorated.



c9 c10 c13

Neutral words against stigmatizing ones

One wording pair in the study set a neutral description against a stigmatizing one. The tape here carries the second, set under the first.

established patient with multiple chronic conditions

17 visits in the past 12 months

17 visits, one line on a record

The stigmatizing line counted 17 visits in the past 12 months. One wristband stands for one visit.



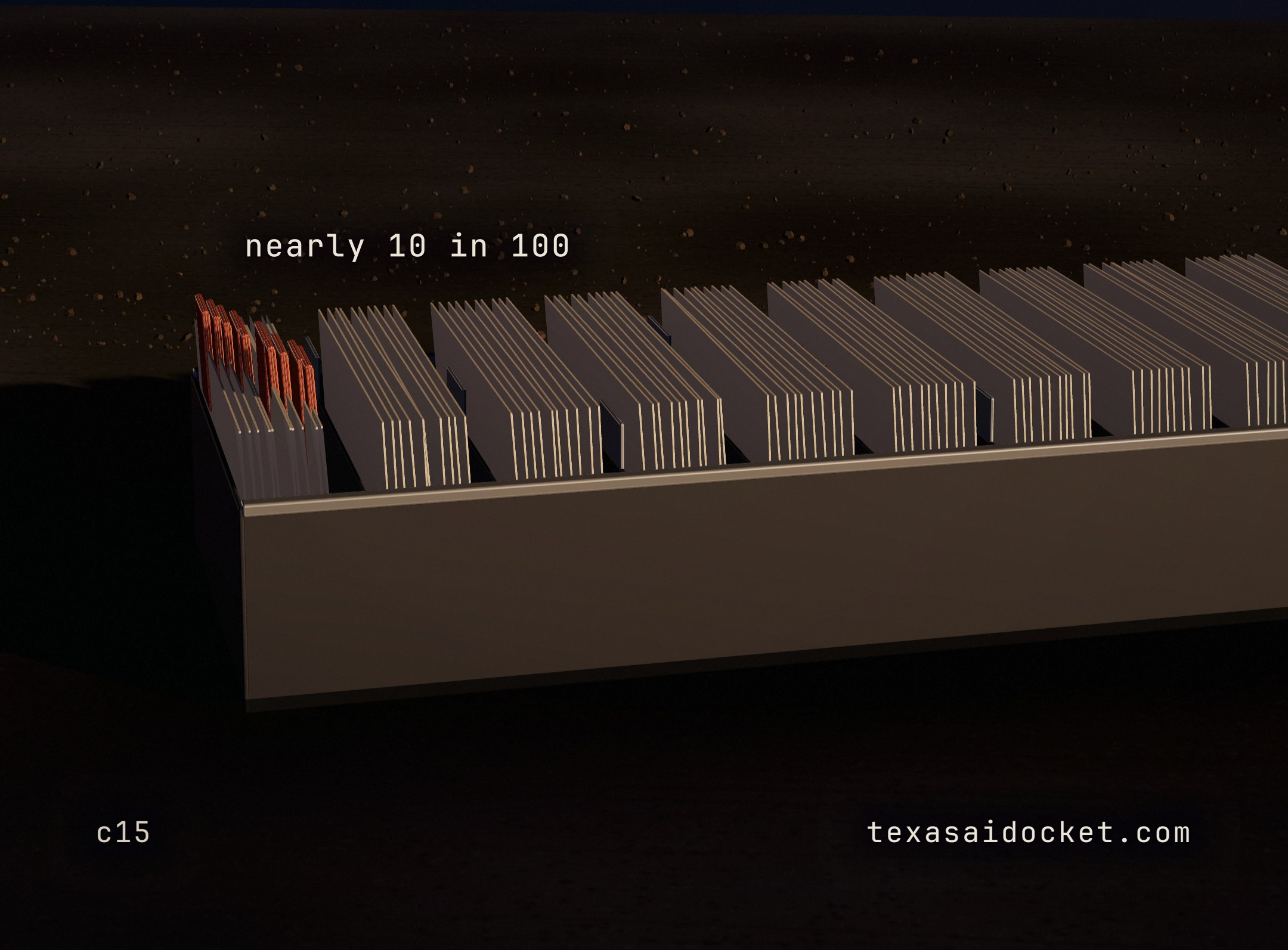
Then the higher risk patient sometimes went second

One added detail sometimes shifted a model away from the patient who later deteriorated, the senior author says. The release does not say how many of the three models shifted.



In some instances, nearly 10% moved back

Nearly 10% of patients were incorrectly downgraded once stigmatizing information was added. The drawer holds 100 cards in blocks of ten, and the flagged block stands for the share.



Five lines stood out in the preprint

In the authors' unreviewed May preprint, the most consistent harmful shifts came from five of 10 added lines. Race, language and insurance produced none. The release names frequent emergency use and past psychiatric emergencies among the strongest effects.



c16 c26 c34 c38 c42

Test each tool on real outcomes first

Bias varies by model, so each AI tool should be tested on real patient outcomes before use, the first author says. UT Southwestern's October 20th release names no Texas hospital using these models for triage.

